



جوستاف روسي انترناشيونال - مصر (EPE) \* طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : سحر صلاح امام فرج

المحافظة : القاهرة الحي / المدينة : خمسة عشر مايو العنوان : 0

رقم البطاقة : 26307200101282 رقم التليفون : 01097095367 تاريخ تسجيل الإستمارة : 04-10-2026

العمر (تاريخ الميلاد) : 20-07-1963 المهنة : ربه منزل

### الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان القولون

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان القولون

خطة العلاج : أورام كيماوي خط علاج كيميائي مابعد الجراحه طبقا لبروتوكول [14236] (Folfox)

التعليق على التشخيص : .....

### تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : ..... التوقيع : .....  
الاسم : د/ندى حسن الوظيفة : ..... التوقيع : .....  
الاسم : د/نرمين رمضان الوظيفة : ..... التوقيع : .....

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

### أعضاء اللجنة

أ.د. / ..... / د. / ..... / د. / .....

### قرار اللجنة

التشخيص النهائي : .....  
خطة العلاج : .....  
المستشفى : ..... قسم : .....  
جهة العلاج : ..... نفقات : .....  
ملاحظات : .....

ختم  
شعار الجمهورية

مدير عام  
مديرية الشؤون الصحية

رئيس  
المجلس الطبي المتخصص

|  |
|--|
|  |
|--|

|                                                                      |                                                                                     |
|----------------------------------------------------------------------|-------------------------------------------------------------------------------------|
| <b>Name:</b> سحر صلاح امام فرح<br><b>National ID:</b> 26307200101282 |  |
|----------------------------------------------------------------------|-------------------------------------------------------------------------------------|

|           |
|-----------|
| تقرير طبي |
|-----------|

|                  |
|------------------|
| السيد            |
| تحية طيبة وبعد,, |

|                       |                       |
|-----------------------|-----------------------|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم:                 | رقم طبي:              |

|                                          |
|------------------------------------------|
| نتشرف بأن نوضح فيما يلي تقريراً عن حالته |
|------------------------------------------|

|                                             |                                             |
|---------------------------------------------|---------------------------------------------|
| تاريخ وساعة الدخول :<br>-- --:-- yyyy-mm-dd | تاريخ وساعة الخروج :<br>PM 01:17 05-07-2026 |
|---------------------------------------------|---------------------------------------------|

|                                        |
|----------------------------------------|
| وبعد الكشف والفحوصات والمتابعة وجد أنه |
|----------------------------------------|

|                                                                                  |
|----------------------------------------------------------------------------------|
| A patient of colon cancer for Adjuvant chemotherapy according to FOLFOX protocol |
|----------------------------------------------------------------------------------|

|                         |                                   |
|-------------------------|-----------------------------------|
| التاريخ :<br>05-07-2026 | الوقت :<br>-- --:--               |
|                         | الطبيب المعالج :<br>د/شيماء لاشين |

جوستاف  
روني  
مدير عام

٩ - ٢٩



مبادرة رئيس الجمهورية  
لدعم صحة المرأة المصرية

الاستشارة  
مصر

09/29/2026

## MDT FORM

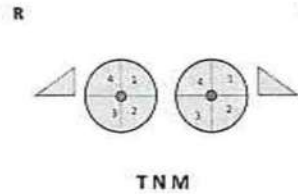
|                                       |                                        |
|---------------------------------------|----------------------------------------|
| Name: <b>محر صلاح امام لرج</b>        | MRN: 14299                             |
| Age: 63 Y                             | ID: 26307200101282                     |
| Menopausal Sta: <b>اخست</b>           | Phone: * 01097095367                   |
| NRS SCORE:                            | Marital Status:                        |
| BMI:                                  | Family History:                        |
| Gender:                               | Reason Of MDT:                         |
| CO Morbidity:                         | new case                               |
| Surgical History: * Cesarian sections | ECOG Performance Status<br>0 0 1 2 3 4 |

## Clinical Data:

BX:   
ER:  PR:  HER2:  KI67:

## History:

Symptoms started 7 months ago by weakness  
Colonoscopy 22/8: sigmoid colon mass 30 cm from anal verge  
Biopsy: Adenocarcinoma G2 KRAS/NRAS/BRAF: No mutation  
PET CT 16/8/2026: Sigmoid colon mass + HFL + LN and pulmonary mets  
Portacath inserted  
PS: 2  
For new CT's - Path revision



## Sonomammography:

## Biomarker/Molecular

## Mets Workup:

## Recommendations:

for Folfox - Erbitux

## MDT Outcome &amp; Plan

Folfox - Erbitux after new CT's and path revision  
Start with 25% DR due to PS then escalate

| Right Side |    |    |    | Left Side |    |    |    |
|------------|----|----|----|-----------|----|----|----|
| Q1         | Q2 | Q3 | Q4 | Q1        | Q2 | Q3 | Q4 |
|            |    |    |    |           |    |    |    |

## Clinical Staging:

## Pathological Staging:

## SIGNATURE

| MEDICAL ONCOLOGY  | SURGERY      | RADIOLOGY | PATHOLOGY |
|-------------------|--------------|-----------|-----------|
| د خالد عبد العزيز | د محمد تيراي |           |           |

جمهورية مصر العربية  
بطاقة تحفيظ الشخصية

سحر  
صلاح امام فرج

مجاورة ١٥ - مجموعه ١٢ - عمارة ٤  
خمسه عشر مايو - القاهرة

١٩٦٣/٠١  
٢٦٣.٧٢٠.٠١.١٢٨٢

HY6031285

الهيئة العامة للغذاء والدواء  
 (The General Authority for Food and Drug  
 Control)

الرقم: ٢٠١٩/٠٣  
 التاريخ: ٢٠١٩/٠٣/١٦

الهيئة العامة للغذاء والدواء  
 (The General Authority for Food and Drug  
 Control)



٢٠١٩/٠٣ ٢٦٣٠٧٢٠٠١٠١٢٨٢

حاصله على بكالوريوس زراعه

مسلمة أنثى

البطاقة سارية حتى ٢٠٢٦/٠٣/١٦





| الاسم        | الجنس | الرقم   | التاريخ    |
|--------------|-------|---------|------------|
| أحمد محمد    | ذكر   | ٢٠١٩/٠٣ | ٢٠١٩/٠٣/١٦ |
| فاطمة أحمد   | أنثى  | ٢٠١٩/٠٣ | ٢٠١٩/٠٣/١٦ |
| عبدالله محمد | ذكر   | ٢٠١٩/٠٣ | ٢٠١٩/٠٣/١٦ |
| مريم أحمد    | أنثى  | ٢٠١٩/٠٣ | ٢٠١٩/٠٣/١٦ |
| خالد محمد    | ذكر   | ٢٠١٩/٠٣ | ٢٠١٩/٠٣/١٦ |
| سندس أحمد    | أنثى  | ٢٠١٩/٠٣ | ٢٠١٩/٠٣/١٦ |
| علي محمد     | ذكر   | ٢٠١٩/٠٣ | ٢٠١٩/٠٣/١٦ |
| نور أحمد     | أنثى  | ٢٠١٩/٠٣ | ٢٠١٩/٠٣/١٦ |
| يوسف محمد    | ذكر   | ٢٠١٩/٠٣ | ٢٠١٩/٠٣/١٦ |
| هدى أحمد     | أنثى  | ٢٠١٩/٠٣ | ٢٠١٩/٠٣/١٦ |

الهيئة العامة للغذاء والدواء  
 (The General Authority for Food and Drug Control)

Name: Sahar Salah Emam  
Referred by: Prof. Karim Fekry

ID: 5270376  
Date: 16/08/2026

### <sup>18</sup>F-FDG PET/CT examination

#### Clinical Indication:

- Patient presented with history of sigmoid colon mural thickening and hepatic lesions.

**Administered activity:** 333 MBq. <sup>18</sup>F-FDG

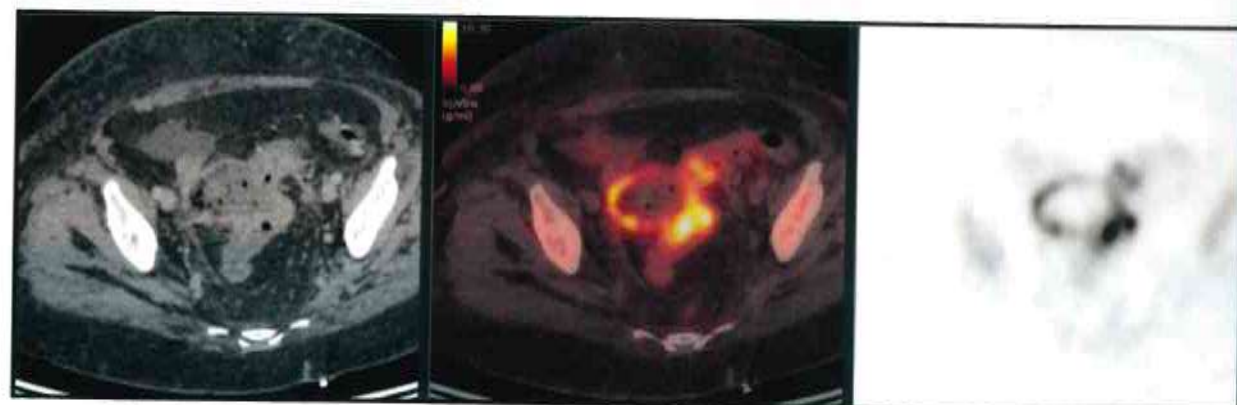
**B.G.L. :** 122 mg/dL

#### Examination protocol:

- A Whole body PET/CT study was obtained from skull vertex to the mid thigh using a low dose non contrast CT for attenuation correction
- Diagnostic multislice CT examination after IV contrast administration of the same region.

#### I. PET/CT Findings:

##### • Colon:



- Increased FDG uptake corresponding to irregular mural thickening of the sigmoid colon, SUVmax 12.1 and measures 2.4 cm in maximal thickness.



- Marginal increased FDG activity related to focal irregular dilatation/collection of the right wall of sigmoid colon with air loculi inseparable from uterus, right adnexa and adjacent small bowel loops, SUVmax 9.8 and measures about 5.1x4.5 cm.

- Stranding of the surrounding fat planes.
- No proximal obstruction.
- Multiple colon diverticulae.
- Mild partly encysted pelvic fluid in between bowel loops.

##### • Lymph nodes:

- Low grade FDG activity within small pericolic nodes, SUVmax 1.7 over subcentimetric node.

##### • Liver:

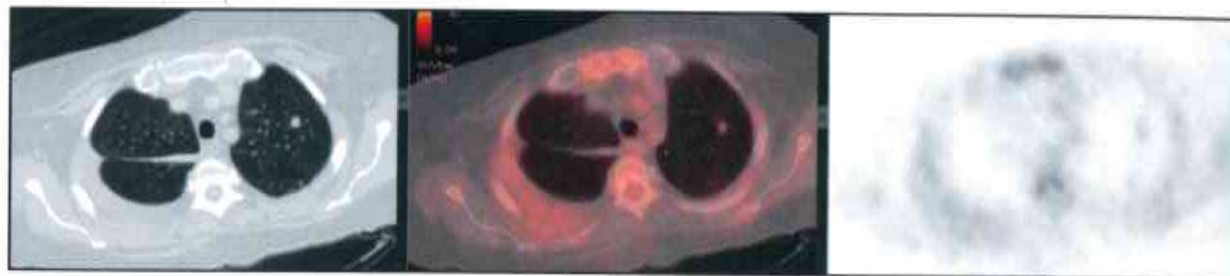


- Enlarged measures about 20 cm in right midclavicular line with increased FDG activity corresponding to numerous hypodense discrete and amalgamated large lesions, reference examples include
  - Segment VII/VIII lesions with SUVmax 10.2 and measures 12.9x11.3 cm.
  - Segment II lesion with SUVmax 9.8 and measures 6x4.7 cm.

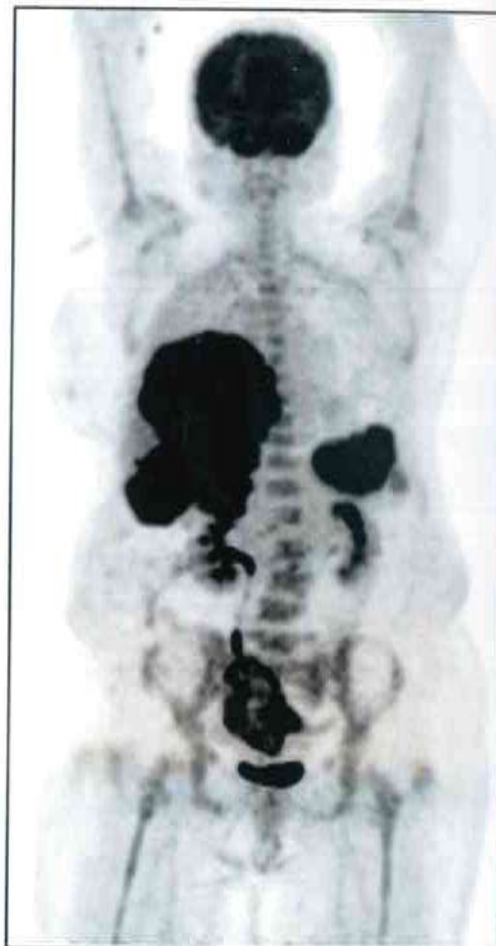


S C A N

• Lungs:



- Low grade FDG activity within multiple left pulmonary nodules, SUVmax 1.2 over 8 mm upper lobe nodule.
- Elevated right diaphragmatic copula with near total collapse of the right middle and lower lobes.
- Low grade FDG activity within right pulmonary atelectatic bands.
- Bone marrow:
  - No FDG avid lytic or sclerotic lesions.
  - Diffuse increased bone marrow FDG activity for clinical and lab correlation.
  - Degenerative changes of the spine are noted.
  - FDG activity related to hips, **inflammatory.**
- Low grade FDG activity within abdominal wall subcutaneous soft tissue stranding associated with non-FDG avid left anterior abdominal wall calcified nodule measures 1.5, **for clinical correlation.**
- Brain: no FDG avid focal lesions.



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II. Diagnostic CT Findings:

- No left pleural, pericardial collections.
- The spleen, pancreas, adrenals, kidneys, aorta and IVC appear unremarkable.
- Intrauterine contraceptive device.
- **Mild right pleural effusion.**
- **Gall bladder stone.**

OPINION:

- Metabolically active sigmoid colon irregular mural thickening associated with right lateral wall dilatation/collection as described, likely neoplastic, for correlation with histopathology.
- Metabolically active lymph nodal, hepatic and pulmonary lesions as described, likely metastatic.

**Radiologist**  
Dr. Mohamed Shahin, MD

**Nuclear Medicine Physicians**  
Dr. Ahmed Tawakol, MD  
Dr. Asmaa Magdy, MD

|                                                                                   |                                                                         |                                                                                       |
|-----------------------------------------------------------------------------------|-------------------------------------------------------------------------|---------------------------------------------------------------------------------------|
|  | <b>All RAS genetic panel testing</b><br><b>by Real-Time PCR (Q-PCR)</b> | Issue No/Revision No:01/01<br>Date of issue:01/03/2025<br>Date of revision:08/09/2026 |
|                                                                                   | TF 05-08-05                                                             | Page: 1 of 2                                                                          |

|                            |                  |                            |                      |
|----------------------------|------------------|----------------------------|----------------------|
| <b>Patient Name</b>        | Sahar Salah Emam | <b>Diagnosis</b>           | Colorectal cancer    |
| <b>Report No</b>           | 2221             | <b>Referring Physician</b> | Khaled Abdelaziz     |
| <b>Date of Birth/Age</b>   | -                | <b>Sample type</b>         | FFPE Block(226/8/22) |
| <b>Gender</b>              | Female           | <b>Report issue date</b>   | 13/09/2026           |
| <b>Sample receipt date</b> | 06/09/2026       | <b>Sample adequacy</b>     | <i>accepted</i>      |

- **Test(s) requested:**
- KRAS, NRAS and BRAF Gene, Somatic Mutation testing.
- **Sample Type:**
- Section of FFPE tumor, >20% neoplastic cells; no micro dissection performed.
- **SPECIMEN ADEQUACY:**
- Adequate. Estimated tumor cellularity (area used for testing): 20%
- **Result:**
- KRAS gene NM\_004985.5: No mutation detected
- NRAS gene NM\_002524.4: No mutation detected
- BRAF gene NM\_004333.6: No mutation detected

| Gene                             | Exons/codons covered                                                            | Result               | Interpretation                                                                   |
|----------------------------------|---------------------------------------------------------------------------------|----------------------|----------------------------------------------------------------------------------|
| <b>KRAS gene:</b><br>NM_004985.5 | Exon 2 (codons 12 and 13) Exon 3 (codons 59 and 61) Exon 4 (codons 117 and 146) | No mutation detected | Absence of mutation shows better response using Chemotherapy ± Anti-EGFR Therapy |
| <b>NRAS gene:</b><br>NM_002524.4 | Exon 2 (codons 12 and 13) Exon 3 (codons 59 and 61) Exon 4 (codons 117 and 146) | No mutation detected | Absence of mutation shows better response using Chemotherapy ± Anti-EGFR Therapy |
| <b>BRAF gene:</b><br>NM_004333.6 | V600E                                                                           | No mutation detected | Absence of mutation shows better response using Chemotherapy ± Anti-EGFR Therapy |

- **Comment:**

|                     |                      |
|---------------------|----------------------|
| <b>Performed By</b> | <b>Authorized By</b> |
| Dr. Mona Ahmed      | Dr. Sally Elfishawi  |

|                                                                                   |                                                                         |                                                                                       |
|-----------------------------------------------------------------------------------|-------------------------------------------------------------------------|---------------------------------------------------------------------------------------|
|  | <b>All RAS genetic panel testing</b><br><b>by Real-Time PCR (Q-PCR)</b> | Issue No/Revision No:01/01<br>Date of issue:01/03/2025<br>Date of revision:08/09/2026 |
|                                                                                   | TF 05-08-05                                                             | Page: 1 of 2                                                                          |

#### Methodology and Limitations:

- DNA was isolated and amplified by quantitative polymerase chain reaction (qPCR) using NRAS and KRAS CE-IVD kits according to manufacture protocol. results interpreted according to the manufacture user guide. Mutation nomenclature was performed considering the following reference sequences: NM\_004985.4 (KRAS) and NM\_002524.4 (NRAS gene).
- BRAF V600E mutation specific Competitive allele specific TaqMan assay along with its reference wildtype assay were performed for each sample to accurately interpret the results.
- The designed primers used in this method and the analysis pipeline are capable of detecting KRAS codons 12, 13, 60, 61, 117 and 146, NRAS codons 12, 13, 60, 61 and 146, and BRAF codon V600E mutations only.
- Sensitivity of KRAS-G12D, KRAS-G12A, KRAS-G12V, KRAS-G12S, KRAS-G12R, KRAS-G12C, KRAS-G13D, KRAS-G13C, KRAS-Q61L, KRAS-A146, NRAS-G12D, NRAS-G12S, NRAS-G13D, NRAS-G13R, NRAS-G12C, NRAS-G12V, NRAS-G12A, NRAS-G13V, NRAS-Q61L, NRAS-Q61H and NRAS-K117N mutations are 1% in a background of 99% normal DNA at 10 ng sample DNA amount.
- Sensitivity of KRAS-Q61R, KRAS-Q61H, NRAS-Q61R, NRAS-Q61K & NRAS-A146T mutations are 2% in a background of 99% normal DNA at 10 ng sample DNA amount.
- Sensitivity of KRAS- K117N, KRAS- A146T, KRAS- A146V and BRAF V600E mutations are 5% in a background of 99% normal DNA at 10 ng sample DNA amount.
- Exon 3 Mutations A59T & Q61K are not covered with KRAS in the testing Methodology.
- Exon 3 and Exon 4 Mutations A59D & K117N aren't Covered with NRAS in The Methodology.
- Limitation: The used methodology can't detect the rare mutation of BRAF V600E2 (1799\_1800TG>AA), it can detect only BRAF mutation E1 (1799T>A).

#### Disclaimer:

- The result should be considered in conjunction with all other parameters of the individual in consultation with their treating physician.

|                |                     |
|----------------|---------------------|
| Performed By   | Authorized By       |
| Dr. Mona Ahmed | Dr. Sally Elfishawi |

Dr.  
**Soliman Saba**

M.Sc., M.D. Pathology

Prof. Of Pathology

Faculty of Medicine, Cairo University

Fellow, Ohio State University U.S.A.

دكتور  
سليمان سبابا  
أستاذ الباثولوجيا  
كلية طب قصر العيني  
زميل جامعة أوهايو بالولايات المتحدة الأمريكية

Name: سحر صلاح إمام

Age: 63 years

Clinical Diagnosis: Malignant sigmoid colon mass at 30 cm from anal verge.

Nature of specimen: Colonoscopic biopsy.

Referred By: Prof. Dr. KARIM FEKRY.

Date: 24/08/2026

## **PATHOLOGY REPORT**

**GROSS:** Specimen received consists of small irregular fleshy grayish white pieces of tissue; colonoscopic biopsy; submitted totally and serially sectioned.

**MICROSCOPIC:** Serial sections examined from the specimen received revealed pieces of tumour tissue formed of papillary projections and irregular; focally fused; glands with cribriform structures lined by malignant columnar epithelial cells showing high nucleo/cytoplasmic ratio, hyperchromatic or vesicular nuclei with scattered mitotic figures. The stroma separating tumour glands is moderately desmoplastic. Tumour tissue was seen infiltrating the colonic mucosal lamina propria and muscularis mucosa.

**DIAGNOSIS:** Sigmoid colon mass, Colonoscopic biopsy, Infiltrating Adenocarcinoma, Grade II.

Thanks for reference,

  
24/8/2026

9/29/26, 5:41 PM

code:09 -- Name\_A MDT FORM -- Name\_E MDT FORM

جوسناف  
روني  
٨٨

٩-٥٩



مبادرة رئيس الجمهورية  
لدعم صحة المرأة المصرية

مستشفى  
البحرية  
الجديدة

09/29/2026

## MDT FORM

|                                              |                                                                                                                                                               |
|----------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Name: <b>سحر صلاح المبرورج</b>               | MRN: 14299                                                                                                                                                    |
| Age: 63 Y                                    | ID: 26307200101282                                                                                                                                            |
| Menopausal Sta: <b>متقاعد</b>                | Phone: * 01097095367                                                                                                                                          |
| NRS SCORE:                                   | Marital Status:                                                                                                                                               |
| BMI:                                         | Family History:                                                                                                                                               |
| Gender:                                      | Reason Of MDT: <b>new case</b>                                                                                                                                |
| CO Morbidity:                                | ECOG Performance Status<br><input type="radio"/> 0 <input type="radio"/> 1 <input checked="" type="radio"/> 2 <input type="radio"/> 3 <input type="radio"/> 4 |
| Surgical History: * <b>Cesarian sections</b> |                                                                                                                                                               |

## Clinical Data:

BX:

R

L

ER:

PR:

HER2:

KI67:

## History:

Symptoms started 7 months ago by weakness

Colonoscopy 22/8: sigmoid colon mass 30 cm from anal verge

Biopsy: Adenocarcinoma G2 KRAS/NRAS/BRAF: No mutation

PET CT 16/8/2026: Sigmoid colon mass + HFL + LN and pulmonary mets

Portacath inserted

PS: 2

For new CT's - Path revision

T N M

Q1 Right Side Q2 Q3 Q4

Left Side Q1 Q2 Q3

Q4

## Sonomammography:

## Biomarker/Molecular

## Clinical Staging:

## Mets Workup:

## Pathological Staging:

## Recommendations:

for Folfox - Eribtux

## MDT Outcome &amp; Plan

Folfox - Eribtux after new CT's and path revision

Start with 25% DR due to PS then escalate

## SIGNATURE

|                   |                |           |           |
|-------------------|----------------|-----------|-----------|
| MEDICAL ONCOLOGY  | SURGERY        | RADIOLOGY | PATHOLOGY |
| د خالد عبد البريق | د محمد نور دهم |           |           |